



Planned Parenthood advocates show their support for the group at the California Democratic Party Convention in Anaheim on May 31.
Carlin Stiehl/Los Angeles Times/Getty Images



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Gov. Gavin Newsom vowed that California would “continue to be a safe haven for all women seeking reproductive health care services in our state” after the Supreme Court’s 2022 decision that overturned Roe v. Wade and paved the way for states to enact abortion bans.

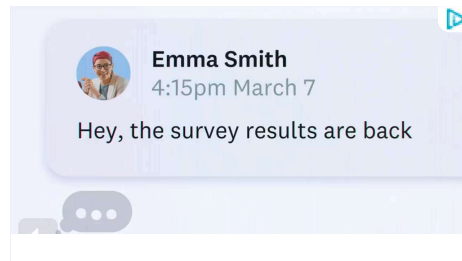
But three years later, as the Trump administration continues its assault on women’s reproductive rights, Newsom’s revised 2025-26 budget hits Planned Parenthood hard and, as written, will imperil the future of the state as a sanctuary.

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If passed, the budget would divert over \$500 million that currently goes to providers like Planned Parenthood to other services. These cuts could reduce Planned Parenthood's budget by up to a third, hindering access to family planning and women's health services. This will lead to fewer clinic hours, longer wait times and potentially shuttering health centers, consequences that would fall hardest on low-income patients, particularly in rural and underserved communities.

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Newsom has pointed to the passage last year of Prop 35, which uses a special tax paid by health insurance plans to fund Medi-Cal provider rate increases and health workforce initiatives, to explain his rationale for diverting funds allocated to reproductive health care. Budget decisions, tough as they may be, reflect priorities. Choosing to balance the budget at the expense of the state's most vulnerable women is morally indefensible and fiscally shortsighted.

Five years ago, I came to work at Planned Parenthood in Santa Rosa because of its mission to ensure compassionate care for everyone who walks through our doors — whether that's an abortion, a cancer screening or birth control. It's why I'm speaking out now: Our patients are at risk of losing that care.

I've seen what happens when people fall through the cracks. I remember a woman with a high-risk pregnancy who, without timely access to care and the trust she placed in us, would have been forced to navigate a far more difficult and potentially dangerous path to access her abortion. We also help patients dealing with domestic violence who are fearful of telling a spouse or family member they are pregnant. These stories are all too common for workers in reproductive health facilities.

As the first-line provider of care for many of our patients, our clinic strives to make sure every need is met, including for patients who, for medical reasons, need to be referred to a hospital, where even routine care can

cost thousands of dollars. Part of my job is to help eligible patients sign up for certain Medi-Cal programs and find funding for women who are ineligible and are uninsured.

Unfortunately, Newsom's budget would also cut Medi-Cal for undocumented adults starting next year, and in 2027, those with "unsatisfactory immigration status" could face unaffordable monthly premiums just to stay insured. This will almost certainly keep many undocumented people from seeking the care they need, which will further compound the negative health impacts of the [\\$880 billion in federal cuts to Medicaid that Republicans in Congress are trying to jam through](#).

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In a single day, a Planned Parenthood clinician can see up to 24 patients for everything from sexually transmitted infection testing, breast exams, pregnancy tests and birth control, to post-abortion care, gender-affirming care and much more. If the governor's budget is approved in its current state, thousands will lose the limited and sometimes only medical care they can access. For many, Planned Parenthood is their first and last line of defense.

The services offered by Planned Parenthood aren't just about reproductive rights; they're about public health and dignity. Upon leaving our clinic, one woman told me, "Thank you for treating me like a human." If clinics are forced to close or cut services, the impact on the lives of women will be profound.

When patients can't access tests for sexually transmitted infections, those infections spread. When they can't get birth control, unplanned pregnancies rise. When care is delayed, outcomes are worse. These are not just strangers who will be affected by this; it affects all of us.

[While the Legislature is pushing back against Newsom's proposed budget cuts, a compromise will ultimately be necessary](#). We need the governor to put the money where his mouth is and ensure the final state budget protects access to reproductive health care for those with the fewest resources — the patients who rely on Planned Parenthood.

Cutting funding from our clinics won't make California safer. It will make people — especially women, LGBTQ+ people, people of color, and low-income communities — more vulnerable and push life-saving care further out of reach.

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If we let this funding disappear, it's not just a blow to Planned Parenthood. It's a blow to public health, and we'll all feel it.

Editor's note: An earlier version of the story misidentified the origin of the \$500 million funding to groups like Planned Parenthood.

Lupita Fernandez is an abortion coordinator/reproductive health specialist IV and SEIU 1021 bargaining team member with the Planned Parenthood Northern California affiliate in Santa Rosa.

June 26, 2025

Lupita Fernandez

More For You

Planned Parenthood Must Choose Between Abortion or Millions in Federal Funding

Under the new spending bill, the nation's largest abortion provider would have to stop offering abortion in some states or lose millions in Medicaid funding for basic health care.



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By Katie Benner, David A. Fahrenthold and Margot Sanger-Katz

July 2, 2025

When Senate Republicans voted on Tuesday to pass President Trump's spending bill, abortion opponents came one step closer to stripping Planned Parenthood of federal funding — a move that could jeopardize abortion access for patients even in states where abortion is legal.

The bill imposes a one-year ban on state Medicaid payments to any health care nonprofit that offers abortions and received more than \$800,000 in federal funding in 2023. The restriction jeopardizes Planned Parenthood's ability to keep operating in some states.

While federal law already prohibits federal Medicaid dollars from paying for abortions, the bill would affect the stream of federal money that covers other services, and thus helps keep clinics open. Though not all individual Planned Parenthood locations provide abortions, the legislative language could knock them out of Medicaid if they are part of a regional network that does offer the procedures.

Planned Parenthood affiliates — each an independently run nonprofit that operates clinics across a geographic region — are in the unusual position of being both an abortion provider and an operator of clinics. The facilities provide a wide range of other medical services for more than two million Americans who cannot afford health care anywhere else.

“This bill threatens to close nearly 200 Planned Parenthood health centers and will create devastating gaps in our health care infrastructure,” Alexis McGill Johnson, the chief executive of Planned Parenthood Federation of America, the national umbrella organization, said in a statement.

The bill most deeply affects Planned Parenthood clinics in blue states, including California, Oregon, Washington and Illinois, where abortion is still legal and where there are also large numbers of patients who are eligible for Medicaid reimbursements for other health care.

Planned Parenthood, whose affiliates already faced dire funding shortfalls in many states even before the latest Medicaid cuts, said in a statement that 200 clinics in 24 states were at risk of closing if the bill passes. Of those clinics, 90 percent are in states where abortion is protected and legal.

The looming funding crisis is one that abortion opponents had wanted for decades.

Republicans have been trying to pass legislation that blocks federal funding to Planned Parenthood for years, but ran into difficulties getting approval from the Senate’s parliamentarian, who determines whether legislation complies with the chamber’s complex rules for the budget process.

This time, to the surprise of many Democratic lawmakers, she blessed the provision. The bill now must be approved by the House of Representatives. The president has said he wants to sign the bill by Friday.

The defunding proposal will not affect states where Republican legislatures have essentially banned abortions. And three states that are hostile to the procedure — Arkansas, Mississippi and Texas — have already blocked Planned Parenthood clinics in their states from receiving Medicaid funding.



“There is not now, and has never been, a plan to stop providing abortion care in the hopes of preserving federal funding,” Alexis McGill Johnson told The New York Times. “Any claims to the contrary are an outright lie.” Eric Lee/The New York Times

Planned Parenthood Federation of America has vowed to continue providing abortions, even in states where funding would be jeopardized by doing so.

That said, it is up to the affiliates, which operate independently from the national organization and are responsible for their own budgets, to evaluate how the bill could affect them and decide how to respond if Congress blocks federal funding.

Medicaid funding accounts for large portions of the operating budgets of many affiliates, and discussions for some of them will be based on how to keep operating without it.

Angela Vasquez-Giroux, a national spokeswoman for Planned Parenthood Federation of America, said that the national office would never recommend that affiliates stop performing abortions. But the national organization “does not direct the actions of an affiliate,” she added. “Our governance structure does not allow for that, by design.”

A decision to halt abortion services by some affiliates could have widespread reverberations. Planned Parenthood is the country's largest abortion provider, yet nearly two thirds of all abortions across the country are provided not by Planned Parenthood, but by hundreds of independent providers.

On the one hand, few, if any, of these independent providers generate \$800,000 in revenue on their own, and thus would not be affected by the law. But if some Planned Parenthood affiliates elect to end abortion access, independent providers in those regions could face pressure to take on more customers — an influx that could quickly overwhelm many of them.

One of the largest affiliates, Planned Parenthood Mar Monte in Fresno, Calif., offers a window into the difficult choice that some clinic operators could face if Mr. Trump signs the spending bill into law. The Mar Monte affiliate runs 35 health centers that serve more than 300,000 patients in mid-California and Nevada.

Mar Monte is one of dozens of Planned Parenthood affiliates that have had to take on a new stream of abortion patients from out of state following the 2022 Supreme Court decision in *Dobbs v. Jackson Women's Health Organization* that ended federal protection for abortion. Following the *Dobbs* ruling, 19 states essentially banned abortion, and clinics in states like California have picked up the slack.

A memo prepared for Mar Monte's leadership in June suggests that the affiliate considered a range of options, including ending abortion.

By offering abortions, the affiliate jeopardizes a good chunk of its revenue. Over the past 12 months, the affiliate estimates, federal Medicaid payments accounted for \$70 million to \$75 million — or nearly half — of its \$152 million in revenue, the affiliate said in an email to The New York Times, and paid nearly 85 percent of Mar Monte's patients' bills.

Those revenues covered services such as reproductive health care, birth control, sexually transmitted infection testing and treatment, breast and cervical cancer screening, pediatric and adult family medicine, gender-affirming care, behavioral health services and prenatal care.

A decision to continue performing abortions, the memo said, would most likely lead to a majority of the patients who rely on Medicaid — about 80 percent of the affiliate's patient base — “no longer being able to access health care.”

It said that preserving abortion access would probably force the affiliate to close 10 health centers and lay off “up to half of our staff.”

On the other hand, the Mar Monte affiliate provided 28,475 abortions in the fiscal year that ended in June of 2024, some to women who could not obtain the procedure in other states.

Stacy Cross, the president of Planned Parenthood Mar Monte, said in an email to The Times that the affiliate evaluated the various scenarios and decided that it would continue to offer abortions even if the spending bill becomes law.

Planned Parenthood's national leadership said there had never been any serious discussion of ending abortion at Planned Parenthood clinics. The Mar Monte discussions were “robust scenario planning” and nothing more, said Ms. Vasquez-Giroux.

“There is not now, and has never been, a plan to stop providing abortion care in the hopes of preserving federal funding,” Ms. McGill Johnson said in a statement. “Any claims to the contrary are an outright lie.”

As of last month, executives at Planned Parenthood Mar Monte were holding out hope for a third option: “A dedicated and generous philanthropist who is willing to make an extraordinary impact that will prevent either of these scenarios from transpiring.”

Leaders estimated in their memo that it would take \$100 million for Mar Monte and \$300 million for all seven affiliates in California to “avert this public health nightmare.”

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